

Dentistry Study Reference

Quiz concepts and source-page map

Study	Quiz 10	Test 25
Review concepts	Short practice	Full mixed test

Built from Dentistry_Study_Guide.md, Master_Dentistry_Glossary.md, and Dental_Pulse_Glossary.md in the supplied Downloads folder. Original textbook PDF page references are retained throughout.

Periodontology

Master Dentistry source pages 15-74

Healthy sulcus	A healthy gingival sulcus is typically 0.5-3.0 mm deep. Source PDF p.15.
Biologic width	The epithelial and connective-tissue attachment is about 2 mm; restorative margins should respect this attachment. Source PDF pp.16 and 60.
Periodontal pocket	A true pocket includes attachment loss. A false pocket is caused by gingival enlargement without attachment loss. Source PDF p.26.
Furcation involvement	Class I furcation involvement is an initial lesion extending no more than 3 mm horizontally. Source PDF p.22.
Aggressive periodontitis	Key features include rapid attachment loss, rapid bone destruction, and familial aggregation. Source PDF p.28.
Scaling and root planing	SRP removes plaque, calculus, and contaminated or necrotic cementum from root surfaces. Source PDF pp.15 and 55.

Endodontics and Restorative Dentistry

Master Dentistry source pages 75-134

Reversible pulpitis	Pain stops soon after the stimulus is removed. Lingering or spontaneous pain is more consistent with irreversible pulpitis. Source PDF p.76.
Root canal access	Straight-line access reduces instrument bending and helps instruments reach the apical portion of the canal. Source PDF pp.82-84.
Sodium hypochlorite	NaOCl is used as an irrigant for its antimicrobial and tissue-dissolving properties. Source PDF p.84.
Patency file	A small file can help prevent apical blockage and maintain canal patency. Source PDF p.85.
MTA	Mineral trioxide aggregate is used for repair procedures and root-end filling. Source PDF p.99.
Glass-ionomer cement	Glass ionomer is moisture-sensitive during setting and also vulnerable to desiccation, so the surface must be protected. Source PDF p.117.
Fissure sealant	Sealants protect susceptible pits and fissures when radiographic dentinal caries is absent. Source PDF pp.187-188.

Prosthodontics and Sedation

Master Dentistry source pages 135-178

Kennedy Class I	Bilateral posterior edentulous areas create bilateral free-end saddles. Source PDF p.147.
Kennedy Class III	A unilateral edentulous area bounded by teeth anteriorly and posteriorly is Class III. Source PDF p.147.
Overdenture	An overdenture covers and gains support from retained teeth, roots, or implants. Source PDF pp.146-147.
Altered cast	The altered cast technique compensates for the different support provided by abutment teeth and mucosa in distal-extension cases. Source PDF p.151.
Inhalation sedation	Nitrous oxide with oxygen provides rapid-onset conscious sedation while the patient remains responsive. Source PDF p.166.
Flumazenil	Flumazenil is a benzodiazepine antagonist used as a reversal agent. Source PDF pp.164-165.

Pediatric Dentistry and Trauma

Master Dentistry source pages 179-236

Pulpotomy	A pulpotomy removes inflamed coronal pulp while preserving radicular pulp tissue. Source PDF p.197.
Pulpectomy	A pulpectomy removes both coronal and radicular pulp tissue before obturation. Source PDF p.199.
Apexification	Apexification creates an apical hard-tissue barrier in an immature non-vital tooth. Source PDF pp.200-201.
Avulsed primary tooth	Replantation is generally not recommended because of the risk to the permanent successor. Source PDF pp.212-214.
Subluxation	Subluxation is loosening of a tooth without displacement. Source PDF p.210.
Dens invaginatus	This developmental invagination commonly affects maxillary lateral incisors and may create a pathway for pulpal infection. Source PDF p.219.
Turner tooth	Localized enamel hypoplasia in a permanent tooth can follow trauma or infection of its primary predecessor. Source PDF p.206.

Orthodontics

Master Dentistry source pages 237-334

Normal overjet	The reference gives a normal overjet range of approximately 2-4 mm. Source PDF p.242.
IOTN	The Index of Orthodontic Treatment Need includes a dental health component and an aesthetic component. Source PDF pp.243-244.
ANB	ANB equals SNA minus SNB; the reference describes Class I as approximately 2-4 degrees. Source PDF pp.253-254.
Class II division 1	This pattern commonly includes an increased overjet. Source PDF p.280.
Functional appliance	Growth-modifying functional appliances are generally timed around the late mixed dentition when growth remains. Source PDF pp.259 and 318.
Retention	The reference suggests about six months after removable-appliance treatment and at least one year after fixed-appliance treatment. Source PDF pp.259 and 321.

Biostatistics

Dental Pulse source pages 4-22

Standard error	The standard error of the mean is SD divided by the square root of the sample size. Source PDF p.5.
Normal distribution	In a symmetric normal distribution, mean, median, and mode are equal. About 95% of observations fall within two standard deviations. Source PDF pp.5 and 15.
Chi-square	Chi-square is used to test association or differences between proportions in qualitative data. Source PDF pp.4-5.
Student t-test	A paired t-test compares before-and-after measurements in the same group; an independent t-test compares separate groups. Source PDF pp.4-5.
ANOVA	One-way ANOVA compares the means of three or more groups. Source PDF p.5.
Type II error	A Type II error occurs when a false null hypothesis is not rejected. Source PDF pp.4-5.
Sensitivity and specificity	Sensitivity identifies true positives; specificity identifies true negatives. Source PDF p.5.

Epidemiology and Oral Health Indices

Dental Pulse source pages 23-59

Incidence	Incidence counts new cases arising in a defined population over a period of time. Source PDF p.23.
Prevalence	Prevalence describes the proportion of a population affected at a point or during a period. Source PDF p.23.
Case-control study	A case-control study looks backward from disease status and commonly estimates association with an odds ratio. Source PDF pp.23 and 31.
Cohort study	A cohort study follows exposed and unexposed groups over time and can directly calculate relative risk. Source PDF pp.21 and 23.
DMFT	DMFT summarizes decayed, missing due to caries, and filled permanent teeth. Source PDF pp.44-46.
CPITN/CPI	The periodontal index uses a WHO probe to record bleeding, calculus, and periodontal pockets. Source PDF pp.45-46.
OHI-S	The Simplified Oral Hygiene Index measures oral debris and calculus. Source PDF pp.44-45.

Prevention, Fluoride, and Caries Activity

Dental Pulse source pages 60-87

Fluoride dentifrice	The reference identifies approximately 1000 ppm fluoride as a common dentifrice concentration. Source PDF p.60.
Topical fluoride rinses	Examples include 0.05% sodium fluoride daily or 0.2% weekly. Source PDF p.61.
Vipeholm study	The Vipeholm study examined the relationship between sugar consistency/frequency and dental caries. Source PDF pp.64 and 74.
Turku sugar study	The Turku study examined sugar substitutes, including xylitol, and caries. Source PDF p.74.
Snyder test	The Snyder test is a colorimetric caries-activity test based on acid production. Source PDF pp.85-86.
Lactobacillus count	The reference identifies this as useful for field studies and sensitive to changes in diet. Source PDF pp.85-86.
Chlorhexidine	The supplied study notes identify chlorhexidine as a reference antiplaque agent. Verify current concentration and indication before clinical use.

Clinical Records and Study Map

Source map for the supplied Downloads files

SOAP	SOAP stands for Subjective, Objective, Assessment, and Plan. Dental Pulse source PDF p.92.
Master Dentistry self-assessment	Question/answer pairs begin at PDF pp.54/58, 102/105, 128/130, 153/155, 174/176, 203/205, 231/233, 263/266, 297/302, 325/328, and 354/358.
Dental Pulse question bank	Biostatistics pp.4-22; Epidemiology pp.23-43; Indices pp.44-59; Preventive Dentistry pp.60-84; Caries Activity Tests pp.85-87; remaining public-health topics pp.88-164.
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